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CORPORATE PRESENTATION

January 2026

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This presentation concerns drug candidates, some of which are undergoing nonclinical studies and others of which are under clinical investigation, and all of which have not yet been approved for marketing by the U.S. Food and Drug Administration. These drug candidates are currently limited by federal law to investigational use, and no representation is made as to their safety or effectiveness for the purposes for which they are being investigated.

Aligos Development Portfolio

2025: An Execution Year

Candidate	Indication	MOA	Discovery	Nonclinical	Phase 1	Phase 2	Phase 3	Partner
pevifoscorvir sodium (pevy)	Chronic HBV Infection *Monotherapy*	CAM-E						
ALG-055009	Obesity, MASH	THR-β Agonist						
ALG-097558	Covid-19*	Protease Inhibitor						NIH National Institute of Allergy and Infectious Diseases; agile Coronavirus Drug Testing Initiative; KU LEUVEN
TBD	Chronic HBV Infection	ASO[1]						AMOYTOP BIOTECH
TBD	Hepatitis Delta Virus	ASO						

*Our Covid-19 protease inhibitor programs are partly funded ($13.8M USD awarded) by the NIH and NIAID's AViDD Centers for Pathogens of Pandemic Concern program through the MAVDA consortium and our recently awarded NIAID Contract. HBV = hepatitis B virus; CAM-E = capsid assembly modulator (empty); Covid-19 = coronavirus disease of 2019 (SARS-CoV-2); MASH = metabolic dysfunction-associated steatohepatitis; THR-β = thyroid hormone receptor beta. ASO= antisense oligonucleotide. All timelines are approximate and subject to change based on enrollment and operational considerations. [1]Amoytop has China rights only for the ASO candidates. Pevifoscorvir sodium formerly known as ALG-000184.

As of 9/30/25 - Cash, cash equivalents and investments were $99.1M.
Projected runway into Q3 2026 inclusive of the $105M raised in Feb. 2025

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CHRONIC HEPATITIS B VIRUS INFECTION

OVERVIEW

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Chronic Hepatitis B Virus Infection

High Unmet Medical Need

- ~254M people worldwide living with HBV with 1.2M new infections annually in 2022
- 1.08M deaths per year, mostly from cirrhosis and hepatocellular carcinoma; Primary cause of liver cancer worldwide
  - Liver cancer is the 6th most diagnosed form of cancer worldwide, but results in the 3rd most deaths of all cancers[6]
- Opportunity estimated at $6.2B by 2031[2]; Gilead HBV sales of ~$1B in 2024[3] at ~$17k/year[4] for continuous therapy
- Pevifoscorvir sodium has the potential for favorable pricing and payor coverage due to its potential to improve patient outcomes[5]

Data from EASL Clinical Practice Guideline 2025 and WHO 2019.
[1]Data largely unavailable for Latin America. 1-2M estimated for the US.
[2]Data from Allied Market Research.[3]Gilead 2024 10-K Filing.
[4]Redbook, accessed August 2024.[5]Based on initial market research data conducted by third party. [6] Bray, et. al. CA J Clin. 2024.

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Current Treatment Options

Nucleoside/Nucleotide analogs (NAs) & Pegylated Interferon alfa (IFNα)

NAs

- Standard of care for 25+ years
  - Indicated for chronic suppression
    - Monotherapy
    - HBV DNA <LLOQ at Week 48 following treatment
  - **Inadequate:** In a recent study with NAs dosed over 5 years, patients continued to progress[2]
    - 4% of patients developed HCC
    - 5% suffered from liver decompensation (variceal bleeding, hepatorenal syndrome, hepatic encephalopathy, ascites)
    - 1% underwent liver transplantation or died

PEG-IFNα

- Not widely used due to tolerability profile
  - Indicated for functional cure
    - Combination
    - HBsAg < LLOQ ~6 months after a finite treatment regimen
  - **Inadequate:** A 1-year course of PEG-IFNα results in an overall seroclearance rate of 2-3% at the end of treatment[1]

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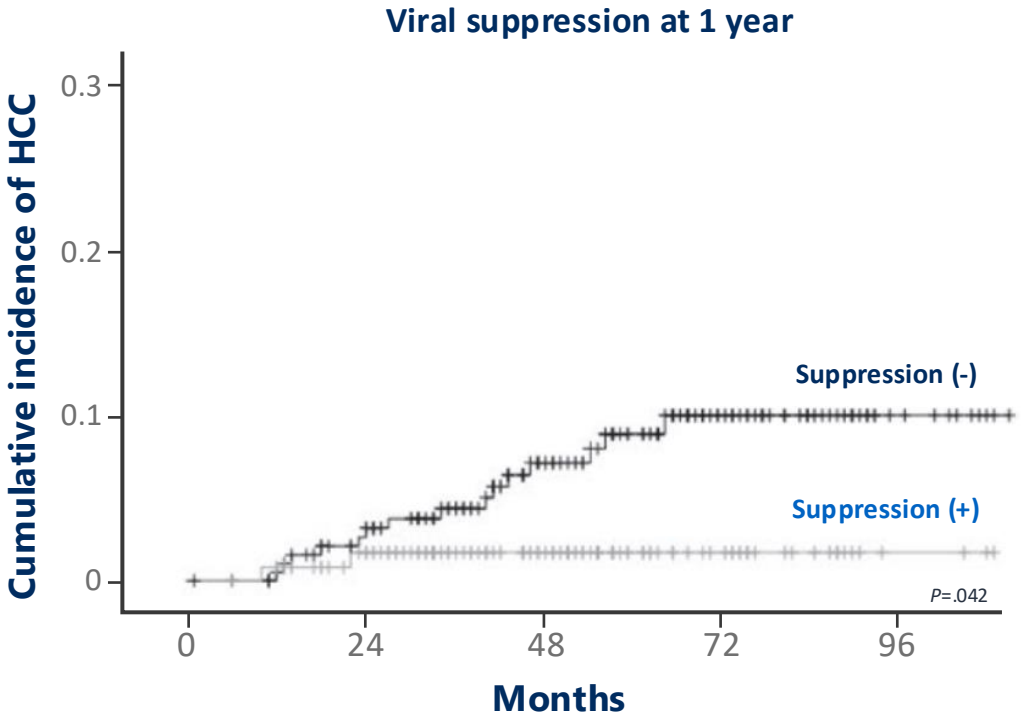
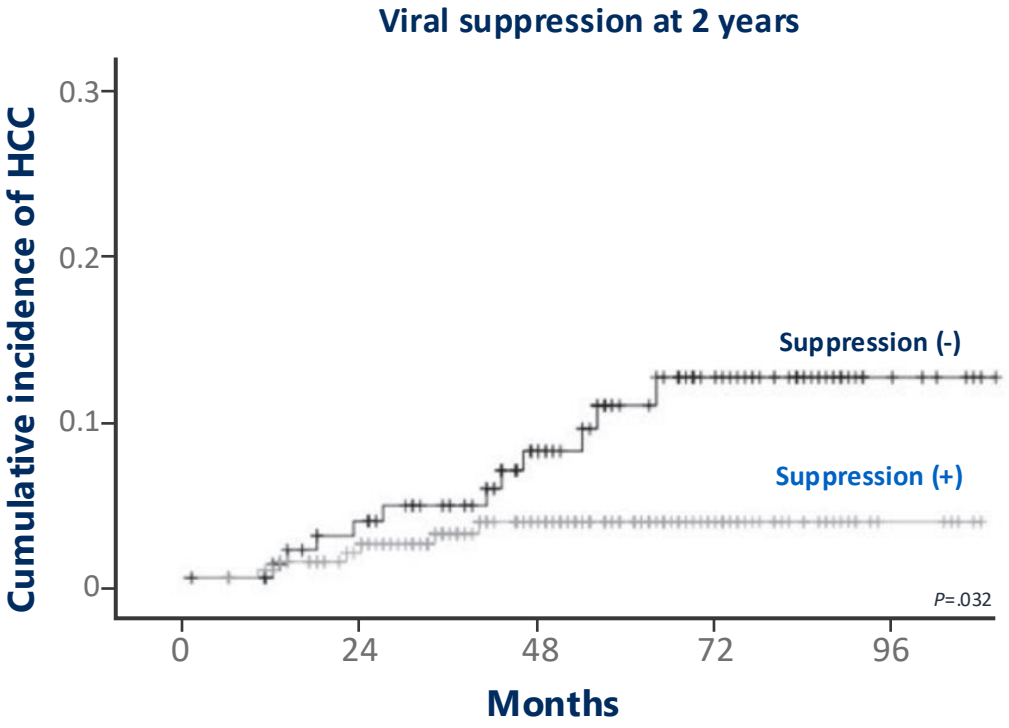
[1]Wong et. al., J Hepatology 2021. [2] Kim et. al., AASLD 2024.

Complete & Faster HBV DNA Suppression Improve Clinical Outcomes



Suppression (-)	201	177	122	54	9
Suppression (+)	124	108	61	23	3



Suppression (-)	123	106	73	38	7
Suppression (+)	202	179	110	39	5

[a] Defined suppression level as 12 IU/mL.[b] Retrospective cohort study of 325 HBeAg-positive patients with chronic HBV infection on antiviral therapy between 2007-2013 in Seoul, Korea. HBeAg, hepatitis B e antigen; HBV DNA, hepatitis B virus DNA; HCC, hepatocellular carcinoma. Nam JY, et al. *J Viral Hepat*. 2018;25(5):552-560.

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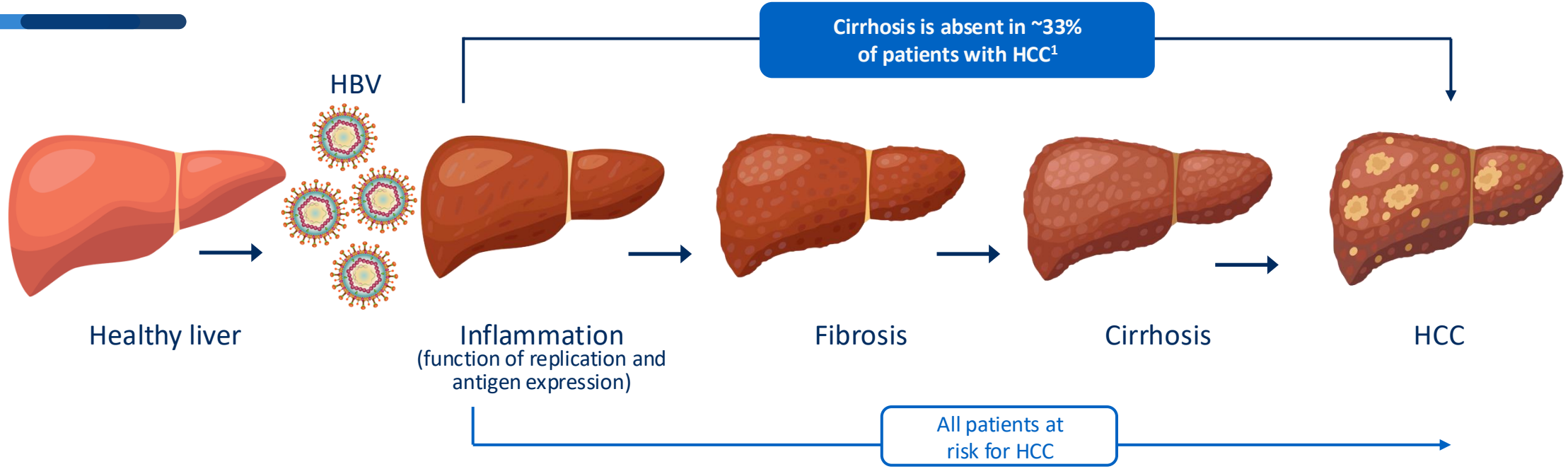
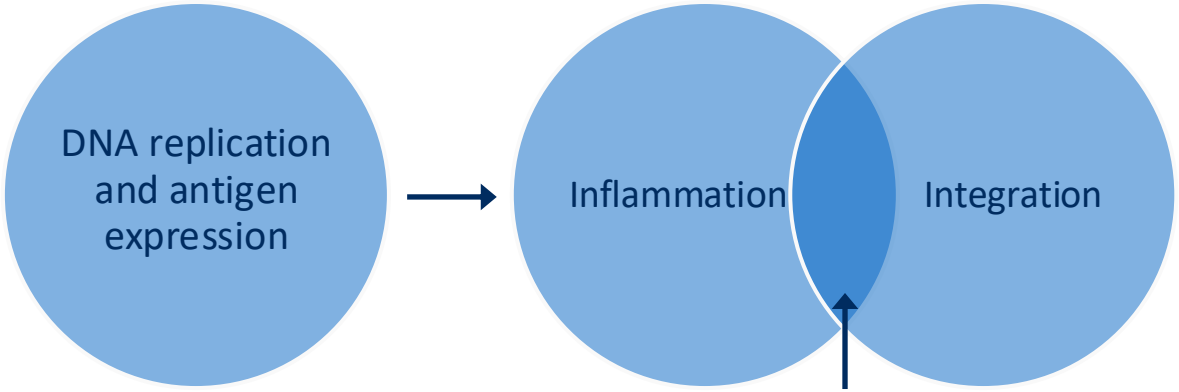
Cumulative HCC and cirrhosis incidence was significantly lower in patients who achieved complete viral suppression[a] within 1-2 years compared with patients who failed to achieve viral suppression within these periods[b]

HBV Pathogenesis: The Importance of Blocking Replication & Antigen Expression



Inhibiting replication and antigen expression, along with blocking the mutagenic effects of integration may translate to reduced inflammation. This has the potential to lead to:

- **A reduction in end-stage liver disease**
- **A reduction in liver cancer**



End-stage liver disease and liver cancer

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[1]Zamor, et. al. JGO, 2017.

Pevifoscorvir Sodium

Small Molecule CAM-E

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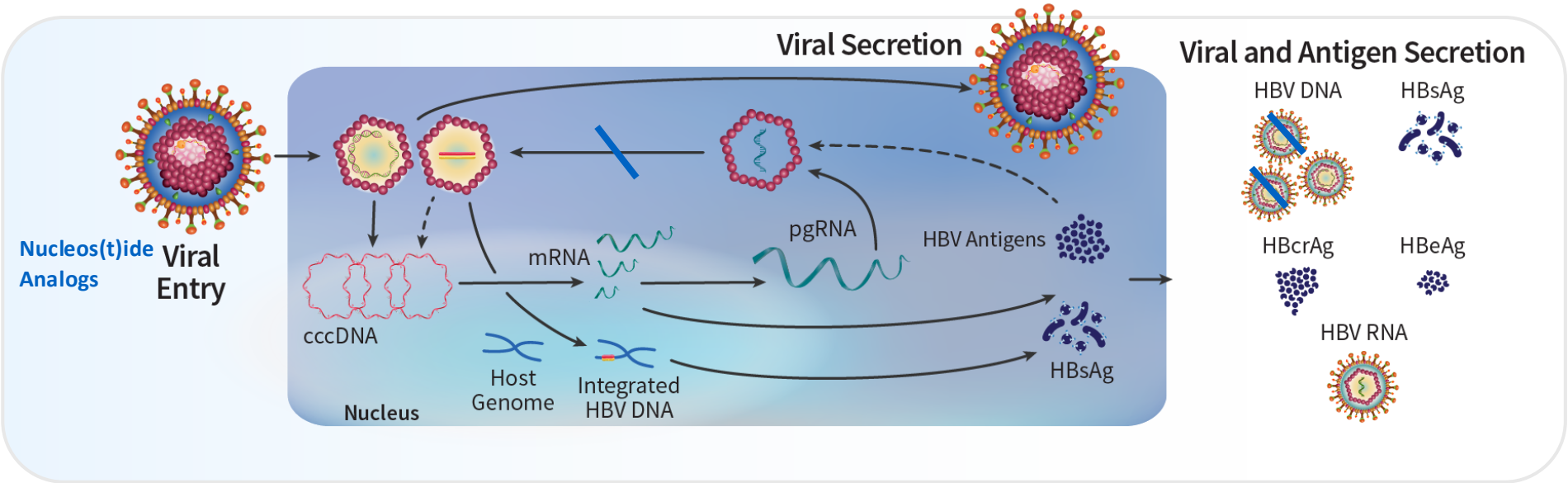
MOA: NAs



Key Attributes

- NAs improve clinical outcomes by controlling HBV replication through targeting HBV polymerase activity, **though they do not[1,2]:**
  - Affect cccDNA[2,3]
  - Substantially lower HBsAg and HBcrAg[3]
  - Completely inhibit rcDNA synthesis from pgRNA[1]
  - Lead to HBeAg seroconversion above the spontaneous rate[4]

1. Martinez MG, et al. *J Hepatol*. 2021;75(3):706-717. 2. Boyd A, et al. *J Hepatol*. 2016;65(4):683-691. 3. Broquetas T, Carrión JA. *Hepat Med*. 2022;14:87-100. 4. Xing T, et al. *PLoS One*. 2017;12(1):e0169444.

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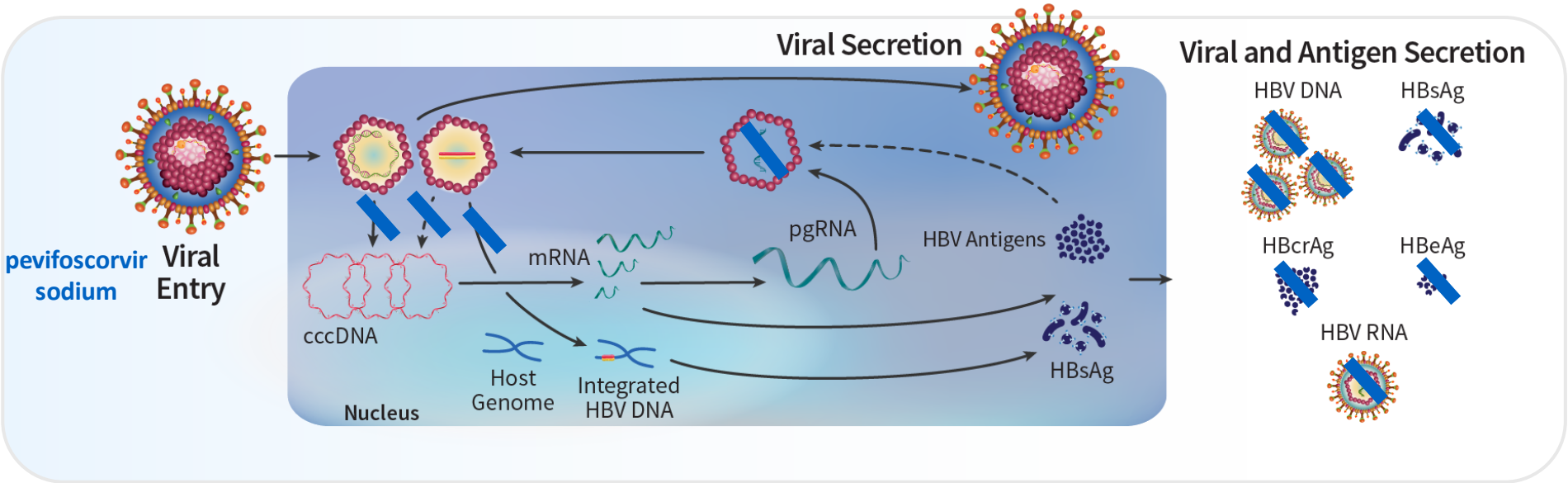
MOA: Pevifoscorvir Sodium (CAM-E)

Suppressing the Entire HBV Lifecycle



Key Attributes

- ✓ **Designed to improve clinical outcomes by causing the formation of empty capsids which reduces viral secretion and prevents capsid disassembly:**
  - ✓ Affects cccDNA
  - ✓ Substantially lowers HBsAg, HBeAg, and HBcrAg
  - ✓ Inhibits rcDNA synthesis from pgRNA
  - ✓ Potentially leads to HBeAg seroconversion above the spontaneous rate

Yuen, et. al, AASLD 2025. Verheyen, et. al, AASLD 2025.

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Pevifoscorvir Sodium: A Favorable Profile

In vitro: potency vs. first generation CAM-Es

Compound	Current Status	HBV DNA reduction (EC_{50} nM)	Cell Type	
Aligos pevifoscorvir sodium[1]	**Phase 2**	**0.63**	**HepG2.117**	2nd Generation
		0.53	**HepG2.2.15**	
Assembly ABI-4334	Phase 1	1.2	AD38	
Assembly ABI-H3733	Partnered (BeOne) – Discontinued	5	AD38	
Enanta EDP-514	Phase 1 – Seeking Collaboration	17	HepG2.115	1st Generation
Assembly ABI-H0731 (vebicorvir)	Discontinued	172	AD38	
Janssen JNJ-56136379 (bersacapavir)	Discontinued	54	HepG2.117	
Arbutus AB-836	Discontinued	10	HepDE19	

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Data was sourced from publicly available literature, posters and presentations; No head-to-head clinical trials have been conducted. Caution should be exercised when comparing data across experiments. [1]ALG-000184 data was generated by Aligos on the parent compound ALG-001075. Studies conducted in vitro.

Pevifoscorvir Sodium (ALG-000184)

A Potential Best-in-Class CAM-E for Chronic Hepatitis B Virus Infection

- **Initial IP licensed from the lab of Dr. Raymond Schinazi at Emory University; issued US patent expires in 2040**[1]
- **Enhanced pharmacology**
  - Picomolar potency with enhanced absorption and high liver uptake
- **Preclinical profile**
  - ~2-300-fold improvement in in vitro potency vs. other known CAMS; superior DMPK properties
- **Phase 1 highlights** (SAD/MAD + QD oral doses (10-300 mg) x 28 days in treatment naïve/currently not treated subjects)
  - **PK:** Dose proportional, low-moderate variability
  - **Safety:** All doses well tolerated
  - **Efficacy**: The lowest dose (10 mg) achieved maximum HBV DNA reductions; more potent antiviral activity than competitor CAMs[2]
  - **MOA:** Evidence of second mechanism seen at 300 mg monotherapy dose through reductions in HBsAg
    - The only CAM to date that has demonstrated HBsAg reduction at 28 days
- **Phase 1:** 96-week dosing completed
- **Phase 2 study enrolling patients:** Planned enrollment (60) completed in HBeAg- cohort; enrollment ongoing in HBeAg+ with >50% (55) of the planned enrollment occurring in January 2026

[1] Not including any patent term extension. [2] Head-to-head data seen in preclinical context.

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ALG-000184-201, A Multi-Part Phase 1 Study of Pevifoscorvir Sodium
Part 4 Cohort Design for Long Term Dosing in TN/CNT Subjects with Chronic HBV Infection

Part 4 Cohort Designs

China

Randomize
4 active:1 placebo

Cohort 2
HBeAg+
N = 15

- Pevifoscorvir sodium 300 mg + ETV x ≤96 weeks → Follow-up (ETV only) x 8 weeks
- Placebo + ETV x 12 weeks → Pevifoscorvir sodium 300 mg + ETV x ≤96 weeks → Follow-up (ETV only) x 8 weeks

ROW

Cohort B
N=21
HBeAg+ n=10
HBeAg- n=11

Pevifoscorvir sodium 300 mg monotherapy x ≤96 weeks (open label) → Follow-up (NA) x 8 weeks

Hou, JL. et al., EASL 2025. Yuen, M-F. et al., EASL 2025.
All cohorts fully enrolled. NCT04536337; ROW: rest of the world.
Note: TN-treatment naïve; CNT-currently not treated.
ETV-entecavir.

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ALG-000184-201

Part 4 Baseline Characteristics

	HBeAg+			HBeAg-
	Part 4 Cohort 2		Part 4 Cohort B	Part 4 Cohort B
	ETV ×12 Weeks followed by 300 mg pevifoscorvir sodium +ETV	300 mg pevifoscorvir sodium +ETV	300 mg pevifoscorvir sodium monotherapy	300 mg pevifoscorvir sodium monotherapy
N	3	12	10	11
Age, years, mean (SD)	28 (4.6)	32.3 (10.2)	34.8 (9.1)	48.5 (12.0)
Female, N (%)	2 (66.7)	6 (50)	3 (30.0)	5 (45.5)
Asian, N (%)	3 (100)	12 (100)	9 (90.0)	3 (27.3)
BMI, kg/m^2, mean (SD)	22.3 (2.8)	22.2 (3.1)	22.4 (2.4)	26.0 (3.5)
HBV Genotype, N (%)	B: 1 (33) C: 2 (67)	B: 4 (33) C: 8 (67)	B: 5 (50), C: 4(40), D: 1 (10)	B:2(18), C:1(9), D:7(64), A:1(9)
HBV DNA, $\log_{10}$ IU/mL, mean (SD)	7.8 (1.1)	8.1 (0.8)	8.0 (0.8)	4.3 (0.7)
HBV RNA, $\log_{10}$ copies/mL, mean (SD)	6.5 (0.9)	6.7 (1.1)	5.3 (1.3)	2.0 (1.0)
HBsAg, $\log_{10}$ IU/mL, mean (SD)	4.1 (0.4)	4.5 (0.7)	4.3 (0.5)	3.5 (0.5)
HBeAg, $\log_{10}$ PEI U/mL, mean (SD)	2.0 (0.3)	2.5 (0.3)	2.6 (0.8)	-
HBcrAg, $\log_{10}$ U/mL, mean (SD)	8.0 (1.2)	8.3 (0.5)	8.3 (0.6)	3.3 (0.6)
ALT, U/L, mean (SD)	38.7 (9.1)	41.5 (22.7)	60.7 (36.9)	35.0 (14.5)

Yuen, M-F. et al; AASLD 2025.

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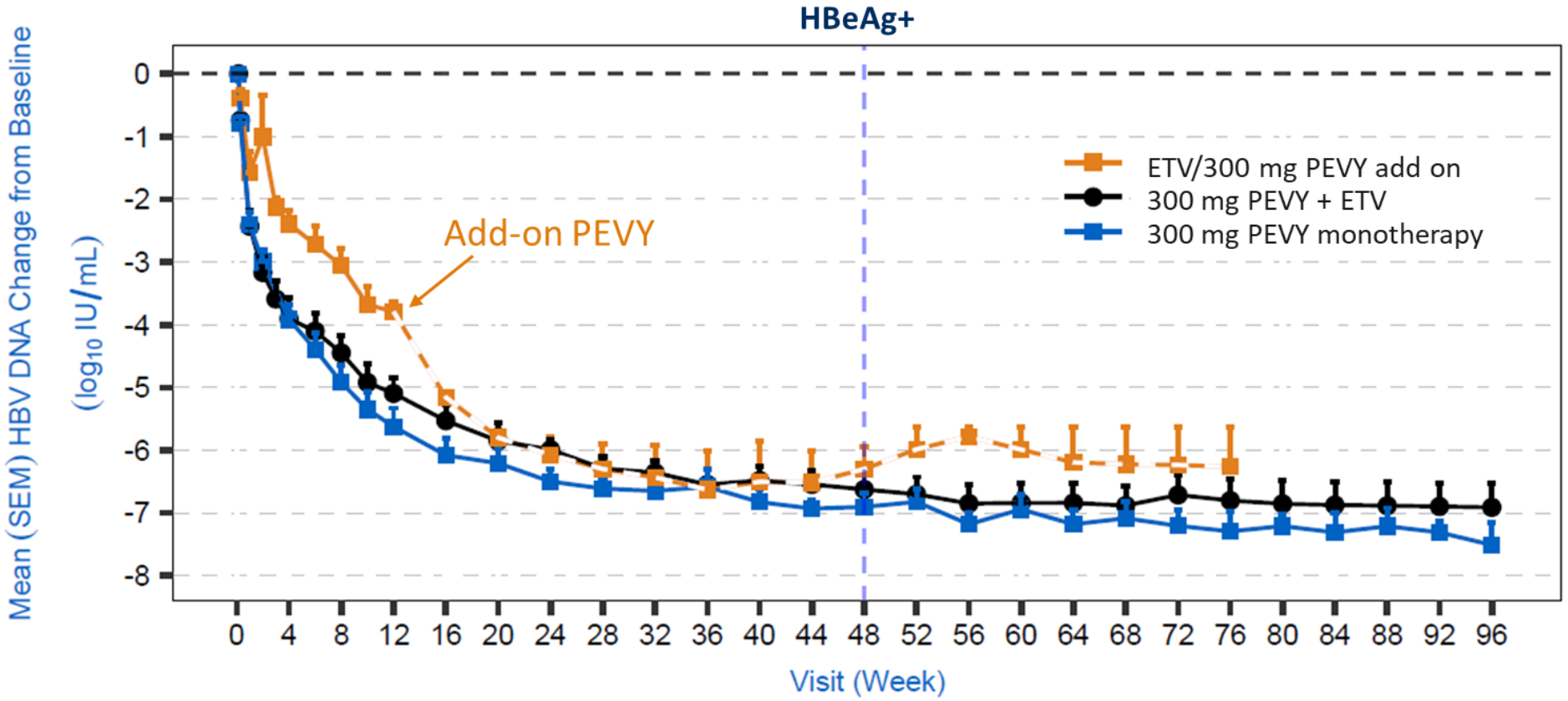
300 mg Pevifoscorvir Sodium ± ETV in HBeAg+ Subjects

Mean HBV DNA Change from Baseline



ETV/300 mg PEVY add on	3	3	3	3	3	2	2		
300 mg PEVY + ETV	11*	9	9	9	8	8	7	7	7
300 mg PEVY monotherapy	10	10	10	10	10	10	10	10	10

Number of subjects at each timepoint

Yuen, M-F. et al; AASLD 2025. *one subject excluded due to dosing non-compliance; PEVY – pevifoscorvir sodium.

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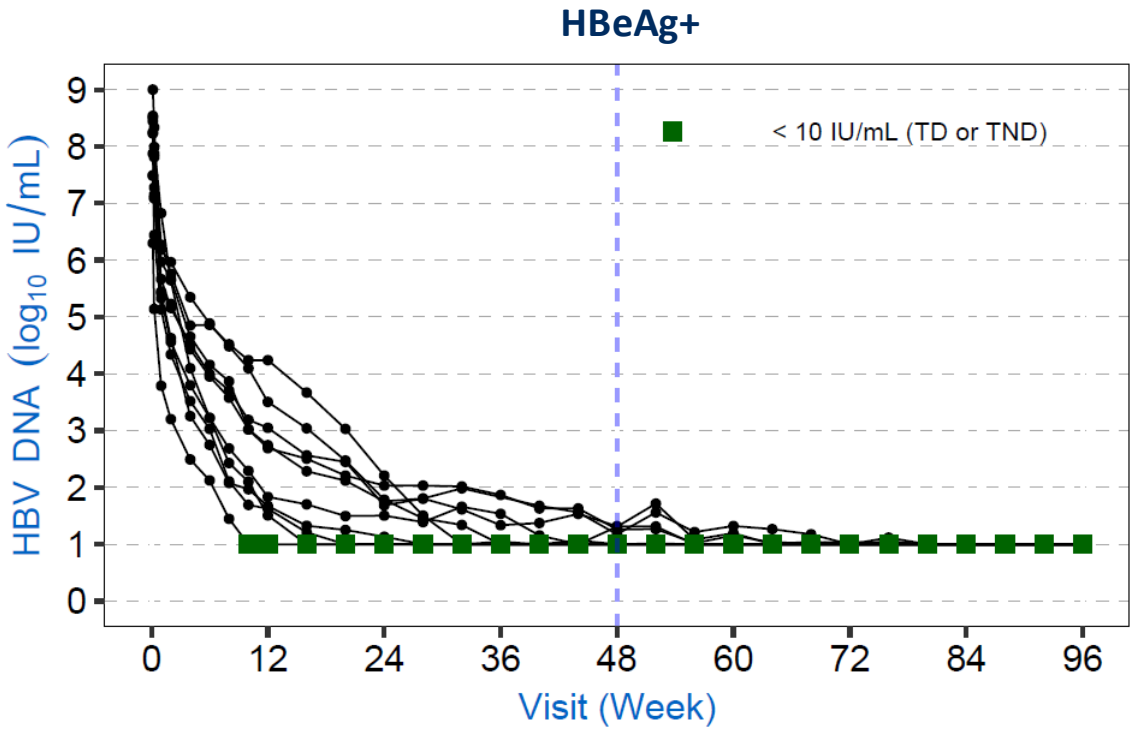
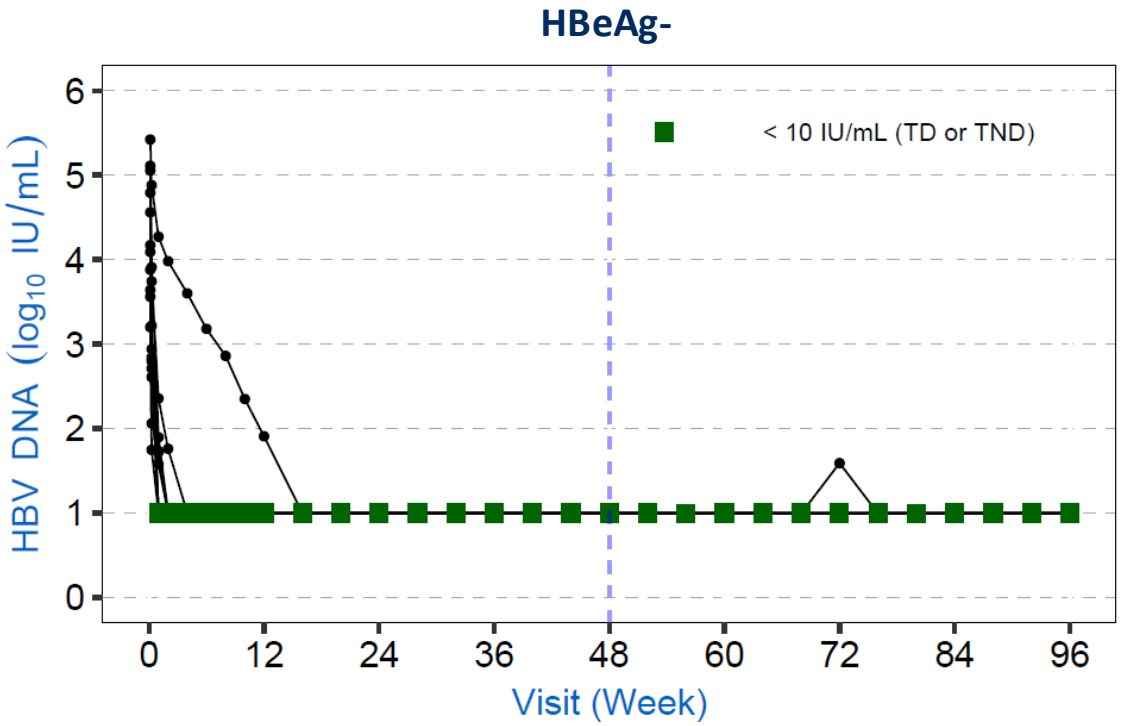
300 mg pevifoscorvir sodium demonstrates greater HBV DNA reduction than ETV monotherapy

300 mg Pevifoscorvir Sodium Monotherapy

Reduction in Individual HBV DNA Levels Over Time





	0	12	24	36	48	60	72	84	96
Total n	10	10	10	9	10	10	10	10	10
< 10 IU/mL (TD or TND)	0	1	2	5	6	7	9	10	10
< 10 IU/mL (TND)	0	0	0	0	0	0	2	3	5

	0	12	24	36	48	60	72	84	96
Total n	11	11	11	11	11	10	9	9	9
< 10 IU/mL (TD or TND)	0	10	11	11	11	10	8	9	9
< 10 IU/mL (TND)	0	7	6	7	10	8	7	8	8

Note: one HBeAg- subject had transient HBV DNA increase from < LLOQ to 39 IU/mL at Week 72. At the subsequent visit (Week 76) HBV DNA was < LLOQ (10 IU/mL).

Yuen, M-F. et al; AASLD 2025. TD – target detected; TND – target not detected, LOD ≤ 4.29 IU/mL.

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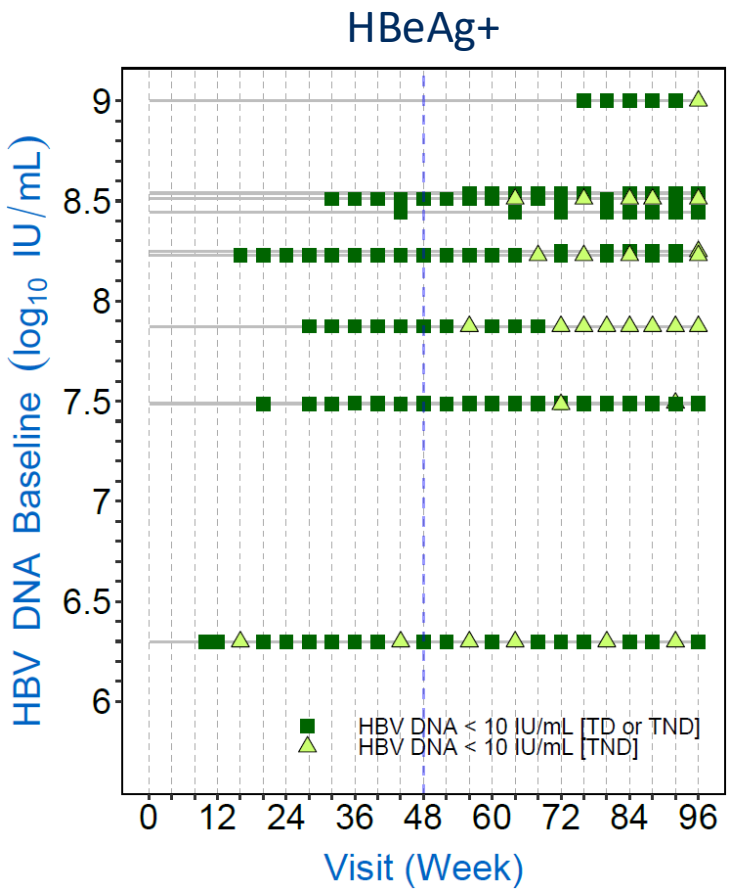
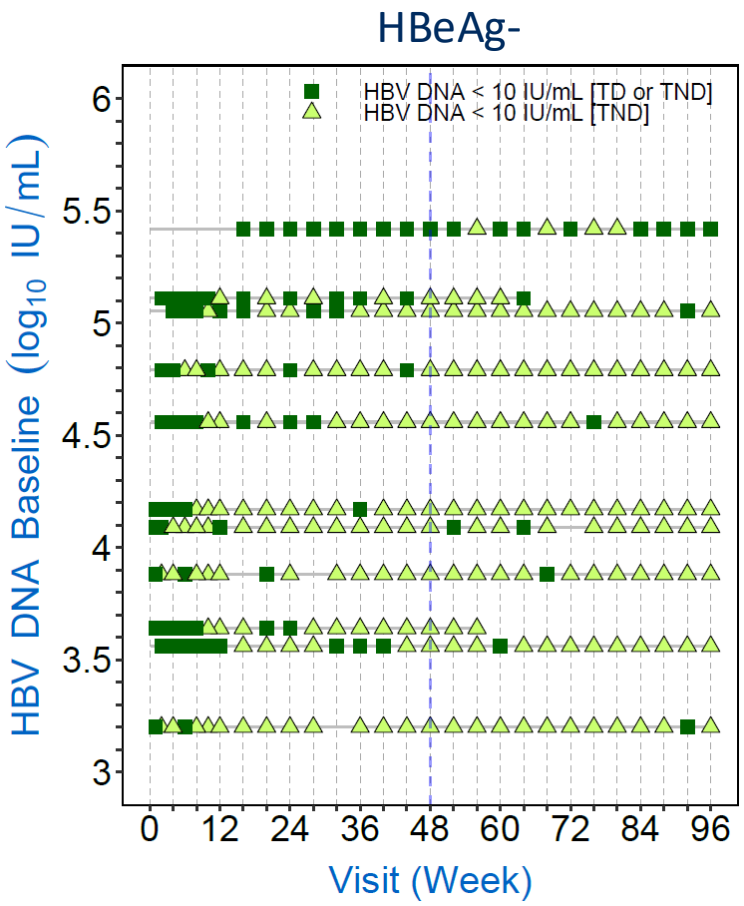
300 mg pevifoscorvir sodium monotherapy demonstrates profound HBV DNA reduction in TN/CNT subjects with chronic HBV infection; no viral breakthrough observed

Antiviral Effect in TN/CNT Subjects with Chronic HBV Infection

Time to Achieve HBV DNA Suppression with 300 mg Pevifoscorvir Sodium Monotherapy





Yuen, M-F. et al; EASL 2025. Data on file. Note: TD-target detected; TND-target not detected.

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The time to achieve HBV DNA suppression (<10 IU/mL [TD or TND]) were dependent on the HBV DNA level at baseline

ALG-001075 In Vitro Resistance Mutation Were Not Detected after 96 W of 300 mg Pevifoscorvir Sodium Monotherapy

Resistance Mutations to other CAMs Detected were observed

Subject	HBeAg Status	Time Point	HBV DNA (IU/mL)	Core Mutation associated with CAM Resistance	Frequency [%]
A	Negative	Day 1	1.23E+04	F23Y	5.10
				I105V	18.33
				Y118F	8.60
B	Negative	Day 1	6.17E+04	I105L	1.98
				I105V	65.88
				T109M	99.82
C	Negative	Day 1	1.48E+04	I105L	11.16
				I105T	6.52
				I105V	30.01
D	Negative	Day 1	1.59E+03	Y38F	97.67
				I105T	99.97
E	Positive	Day 1	2.78E+8	none	n.a.
		Day 42	1.68E+3	D29G	1.06

- Resistance mutations to other CAMs were detected:
  - Bersacapavir: (F23Y, 5x; Y118F, 7x; D29G, 4x)
  - Vebicorvir: (Y118F, 14x; T109M, >68x; Y38F, 3x; D29G, 20x)

Jekle, et. al. EASL 2025. Data was sourced from publicly available literature, posters, and presentations.

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No viral breakthrough or viral non-response based on HBV DNA levels and sequence analysis seen in pevifoscorvir sodium clinical studies to date

300 mg Pevifoscorvir Sodium Monotherapy

Phase 1 Results vs. Standard of Care TDF/TAF

HBeAg Status	Drug	% Patients < LLOQ at Week 48 (by HBV DNA Assay Sensitivity)		% Patients < LLOQ at Week 96 (by HBV DNA Assay Sensitivity)	
		% < LLOQ 29 IU/mL	% < LLOQ/D 10 IU/mL	% < LLOQ 29 IU/mL	% < LLOQ/D 10 IU/mL
E-	TDF (n=140)[a]	93%	17%[f]	91%	31%[f]
	TAF (n=285)[a]	94%	21%[f]	90%	33%[f]
	300 mg pevy (n=11)[d]	100%[4]	100%[4]	100%[5]	100%[1]
E+	TDF (n=292)[b]	67%	N/A	75%	9%
	TAF (n=581)[b]	64%	N/A	73%	14%[e]
	300 mg pevy (n=10)[c]	100%[1]	60%[2]	100%[1]	100%[1]

TAF-tenofovir alafenamide, TDF-tenofovir; LLOQ-lower limit of quantification.
[a] Buti et. al., Lancet Gastro 2016; [b] Chan et. al., Lancet Gastro 2016. [c] Yuen, M-F. et al., AASLD 2025. [e] Kosh A. et al.; Journal of Hepatology 2018 V68: 672-681. [f] LLOQ <29 IU/mL (LLOD <10 IU/mL). [1] 10/10 subjects. [2] 6/10 subjects. [4] 11/11 subjects. [5] 9/9 subjects. [6] No head-to-head clinical trials have been conducted. Caution should be exercised when comparing data across trials. Pevy = pevifoscorvir sodium.

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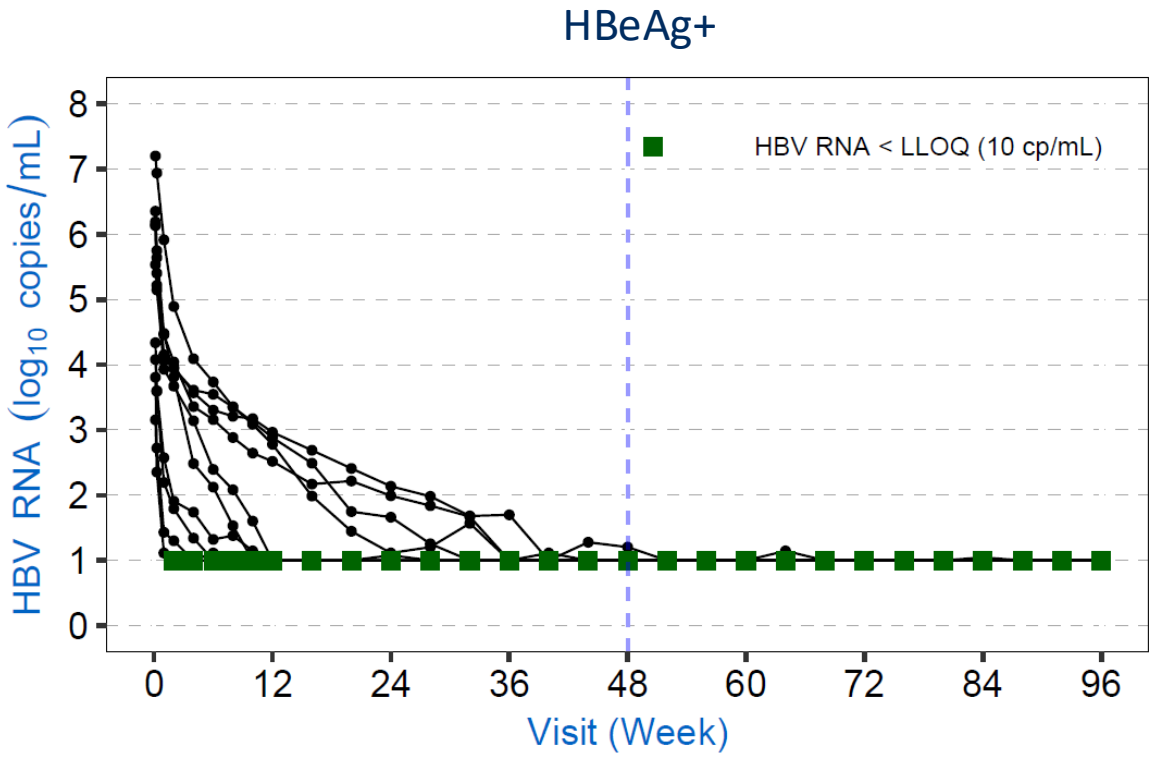
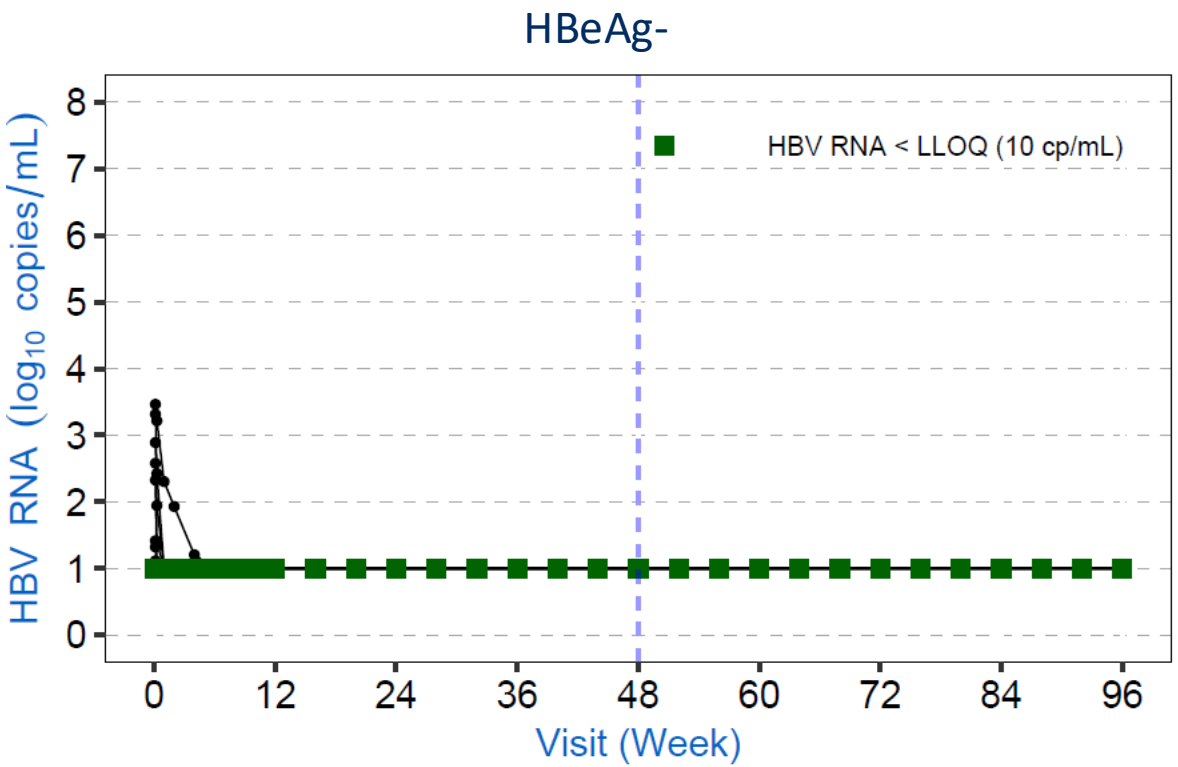
HBV DNA data suggests 300 mg pevifoscorvir sodium monotherapy may achieve superior chronic suppression, which is currently being evaluated in a Phase 2 study

300 mg Pevifoscorvir Sodium Monotherapy

Reduction in Individual HBV RNA Level Over Time





Total n	10	10	10	9	10	10	10	10	10
< 10 cp/mL	0	6	5	8	9	10	10	9	10

11	11	11	11	11	10	9	9	9
3	11	11	11	11	10	9	9	9

Yuen, M-F. et al; AASLD 2025. LLOQ – lower limit of quantitation. cp - copies.

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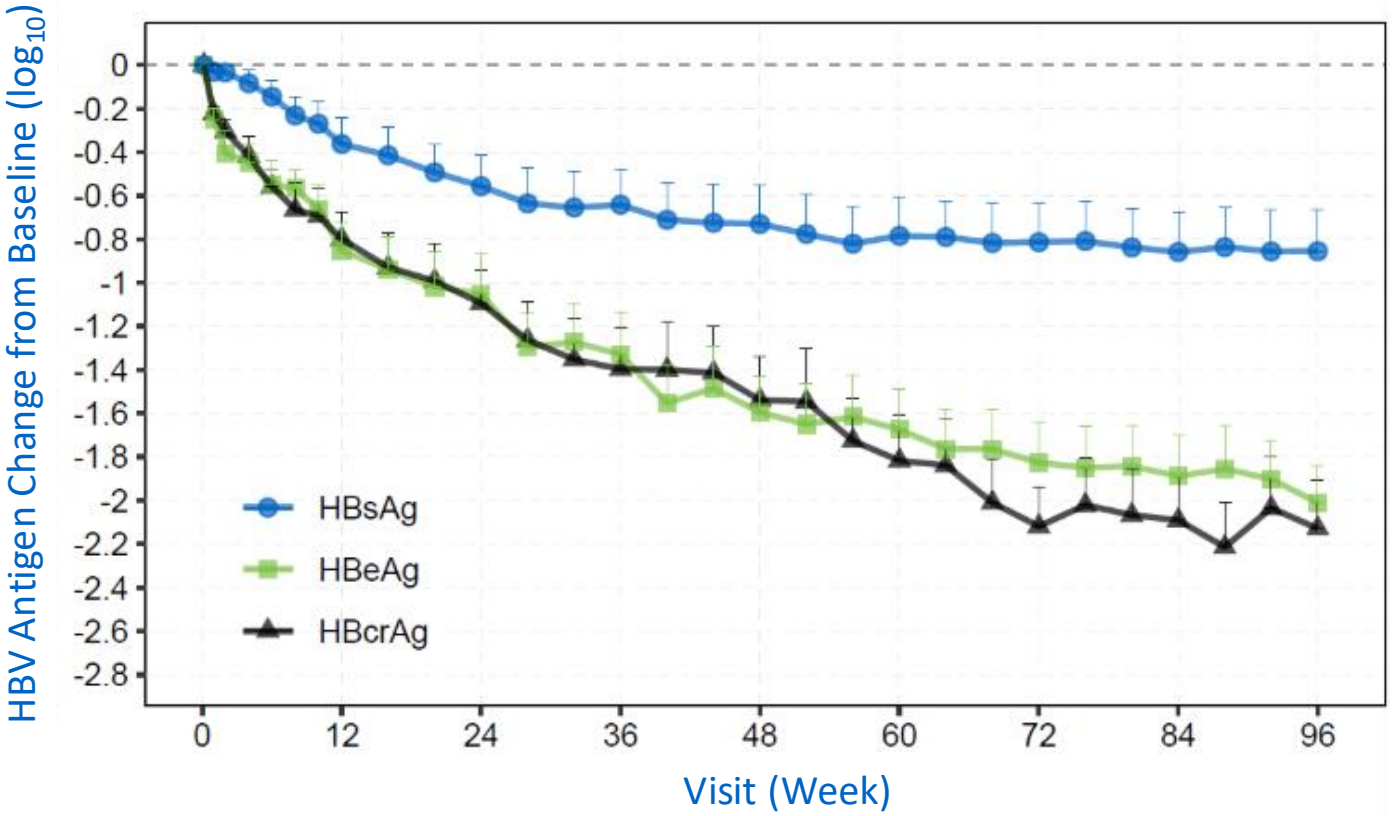
300 mg pevifoscorvir sodium monotherapy demonstrated a rapid and profound reduction in HBV RNA in TN/CNT subjects with chronic HBV infection

300 mg Pevifoscorvir Sodium Monotherapy in HBeAg+ Subjects

Mean HBV Antigen Change From Baseline



	0	12	24	36	48	60	72	84	96
HBsAg	10	10	10	10	10	10	10	10	10
HBeAg	10	10	10	10	10	10	10	10	10
HBcrAg	10	10	10	10	10	10	10	10	10

Yuen, M-F. et al; AASLD 2025. Note: Data represents Mean (SEM) at each visit.

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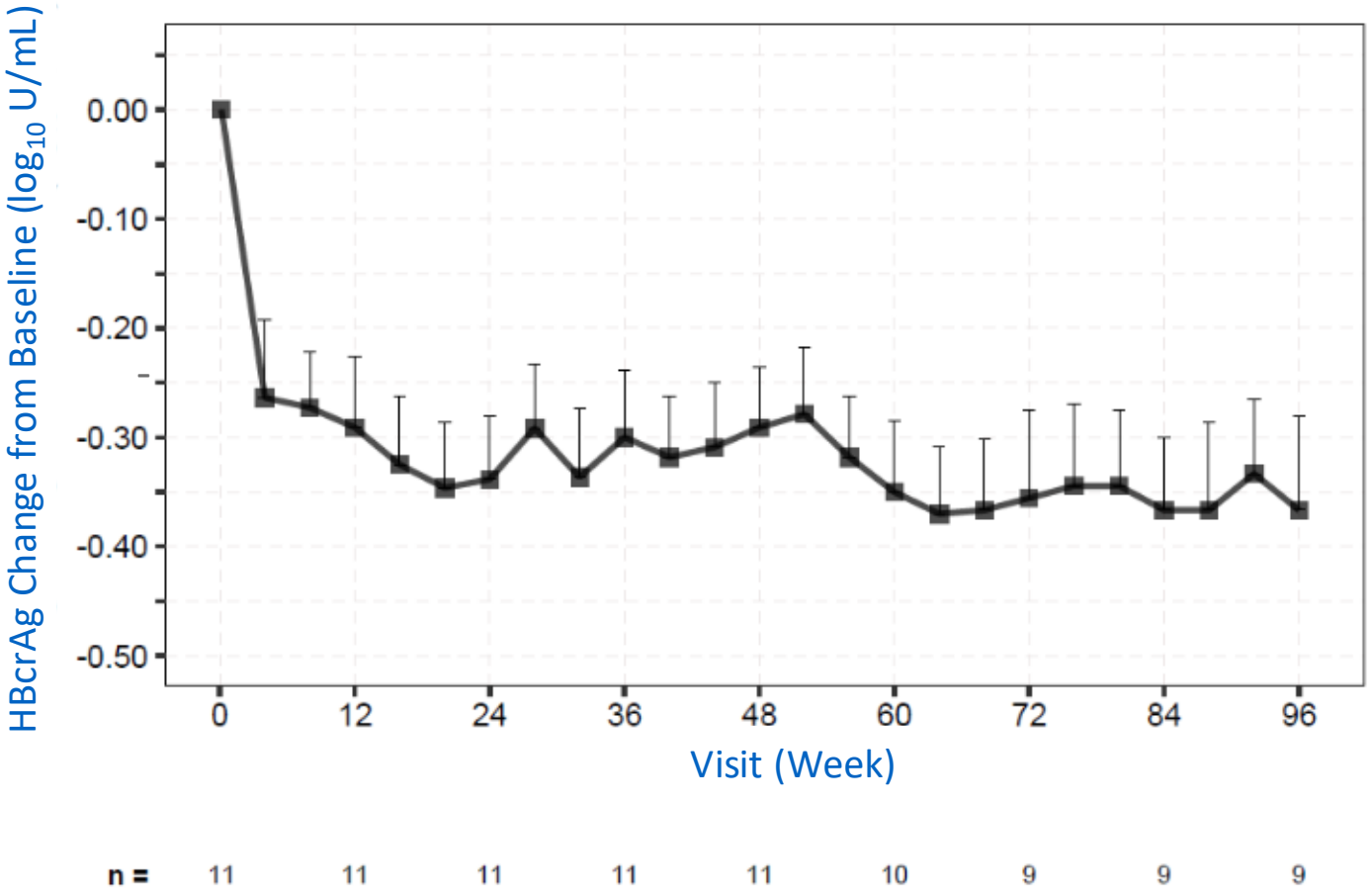
Substantial HBsAg, HBeAg, and HBcrAg reductions noted in HBeAg+ subjects

300 mg Pevifoscorvir Sodium Monotherapy in HBeAg- Subjects

Mean HBcrAg Change From Baseline



Yuen, M-F. et al; AASLD 2025. Data represents Mean (SEM) at each visit.

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Meaningful HBcrAg reduction in HBeAg- subjects, no change in HBsAg

300 mg Pevifoscorvir Sodium Monotherapy

Safety Following 96 Weeks of Treatment

	HBeAg+	HBeAg-
Numbers of subjects with	N=10	N=11^
• **at least one TEAE, n (%)**	9 (90)	9 (81.8)
• **SAE**	0	0
• **TEAE leading to study drug discontinuation**	0	0
• **TEAE Grade ≥3**	3*	2*,#

^ Two HBeAg- subjects withdrew at Week 56 and 64, respectively, due to non-safety personal decisions.
* Grade ≥3 TEAEs of ALT/AST elevation were observed in 3 HBeAg+ and 1 HBeAg- subjects with preserved synthetic and excretory functions. All events resolved in the setting of continued pevifoscorvir sodium dosing and were not considered clinically concerning by the ALT Flare Committee.
# Grade 3 cholesterol/triglycerides increase in HBeAg- subject resolved in the setting of continued pevifoscorvir sodium dosing.

Yuen, M-F. et al; AASLD 2025.

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Favorable safety profile observed in TN/CNT HBeAg+ and HBeAg- subjects with chronic HBV infection who received 300 mg pevifoscorvir sodium monotherapy for 96 weeks

96-Week 300 mg Pevifoscorvir Sodium Monotherapy Post Treatment Data

8-Week NA Follow Up

HBeAg Status	Viral Marker	Data
E-	HBV DNA	8/8 subjects had HBV DNA level < 10 IU/mL during NA only 8 Week follow-up
	HBV RNA	HBV RNA rebounds slightly after switching to NA but remains lower than baseline
	HBV Antigens	HBcrAg decline was maintained during the NA only 8 Week follow-up
E+	HBV DNA	6/8 subjects had HBV DNA level < 10 IU/mL during the NA only 8 Week follow-up
	HBV RNA	HBV RNA rebounds after switching to NA but remains lower than baseline
	HBV Antigens	No apparent HBV antigens increase observed during the NA only 8 Week follow-up

Yuen, et. al. AASLD 2025.

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These data suggest that pevifoscorvir sodium may reduce the cccDNA pool through engagement of its secondary mechanism of action

Pevifoscorvir Sodium

- Regulatory Pathway
- Phase 2 Study Design

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Chronic Suppression

Well Defined, Validated Approval Pathway

- Regulatory pathway for chronic suppressive therapy endorsed by FDA, CHMP (EMA), and National Medical Products Administration in China
- Primary endpoint: Subjects with HBV DNA <LLOQ (10 IU/mL) at Week 48 following treatment with investigational agent versus standard of care (nucleos(t)ide analogs)

a. Chronic suppressive therapy

Sponsors developing drugs for chronic suppressive therapy of CHB should consider the following trial design options:

- A randomized controlled noninferiority (NI) (or superiority) trial comparing the investigational drug with an approved active control arm—The primary efficacy endpoint should be undetectable HBV DNA[13] after 48 weeks on-treatment in HBeAg-positive subjects, HBeAg-negative subjects, or both. The active comparator should be an antiviral drug that is recommended for treatment of CHB and reflects current practice at the time of trial initiation. The patient population may be treatment-naïve or previously treated subjects with detectable HBV DNA. Potential concerns related to the development of resistance when evaluating an investigational drug with a low barrier to resistance as a monotherapy should be addressed. For an NI design, determination of the NI margin should be discussed with the FDA.

HBV Guidance from: FDA 2022; EMA 2006; China 2023; LLOQ: lower limit of quantitation.

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Aligos has received FDA, CHMP (EMA), and National Medical Products Administration in China feedback supporting subsequent studies utilizing this pathway

SUPREME – Phase 2 Study Design

Primary Analysis at 48 Weeks; Extension Period Analysis at 96 Weeks; Interim Readouts Planned

Randomize 1x1

Part 1a
N ~ 110
HBeAg+

Arm 1 N~55	300 mg pevy + Placebo of TDF x 48W	300 mg pevy monotherapy x 48W	pevy long term study or follow up visit
Arm 2 N~55	300 mg TDF + Placebo of pevy x 48W	300 mg pevy monotherapy x 48W	pevy long term study or follow up visit

Primary Endpoint: HBV DNA <LLOQ (10 IU/mL, **target detected *or* target not detected**) at Week 48

Randomize 1x1

Part 2a
N ~ 60
HBeAg-

Arm 1 N~30	300 mg pevy + Placebo of TDF x 48W	300 mg pevy monotherapy x 48W	pevy long term study or follow up visit
Arm 2 N~30	300 mg TDF + Placebo of pevy x 48W	300 mg pevy monotherapy x 48W	pevy long term study or follow up visit

Primary Endpoint: HBV DNA <LLOQ (10 IU/mL, **target not detected**) at Week 48

Part 1b will be a liver biopsy sub-study inclusive of n≈12. Part 2b will be a liver biopsy sub-study inclusive of n≈12. pevy = pevifoscorvir sodium.

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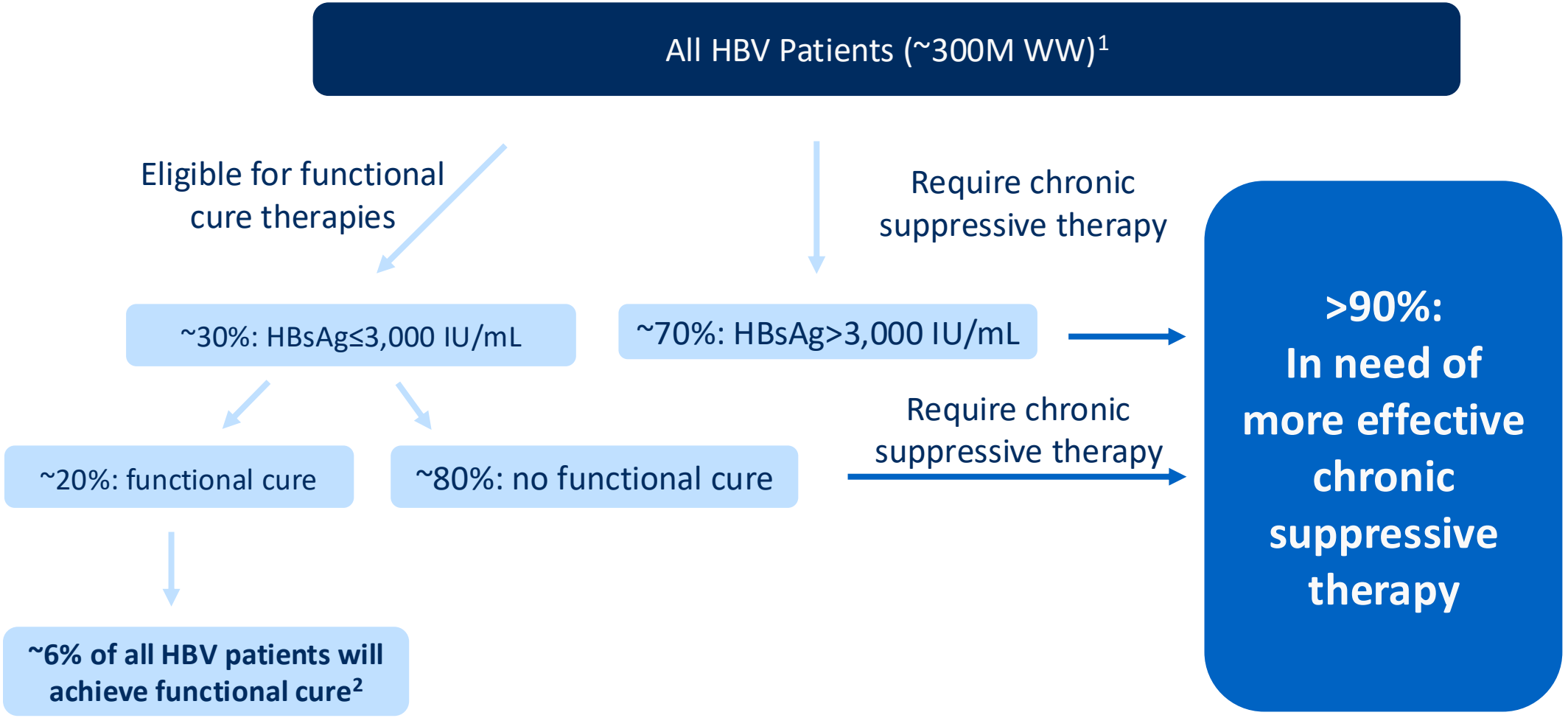
Planned enrollment completed in Part 2a; Part 1a currently enrolling subjects with 50% of the planned enrollment occurring in January 2026

The Future Patient Journey for HBV Therapy



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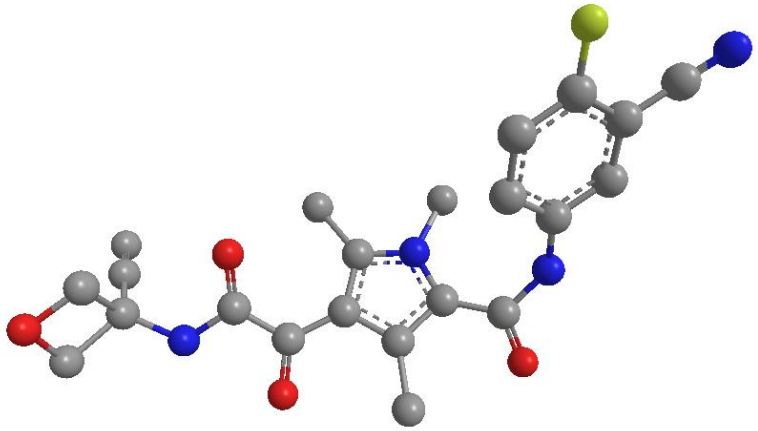
1. Data from the World Health Organization; 2019. Figures are estimated based on feedback from HBV KOLs. 2. Yuen M-F, et al. *N Engl J Med*. 2022;387(21):1957-1968.

Our Vision

Paving the Way for the Future of Chronic HBV Infection Treatment

pevifoscorvir sodium
has the potential to…



- Become standard of care
- Act as the backbone of next-generation therapies, inclusive of in-house programs (ex. ASO)
- Treat HBV patients that may not qualify for current therapies
- Affect the entire HBV lifecycle
- Lower HBsAg, allowing more patients to be eligible for curative therapies

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Payor Perspectives: HBV Remains a Significant Unmet Medical Need

Chronic HBV Infection is the Leading Cause of Liver Transplant Worldwide

Initial market research indicates that payors are potentially willing to pay a premium for drugs that affect the entire HBV lifecycle to mitigate the expense associated with liver disease[4]

Liver disease and liver cancer are costly for payors

- Average cost for a liver transplant is **$1.2M**[1]
- Yearly healthcare costs can exceed **$184k/year**[2] for patients with decompensated cirrhosis
- Cumulative direct costs were estimated at **$45B**[3] in the US over the next 30 years

Faster and more profound reductions in all viral markers of disease have been shown as prognostic indicators for improved outcomes, which were cited by some payors as reasons to cover non-generic options

- Generics already exist, yet certain brand name drugs are still generating ~$1B in revenue per year

Current standard of care drugs are not indicated for all HBV patients, potentially leading to greater healthcare costs

1. Kaplan, et. al. Liver Transpl. 2023. 2. Nguyen, et. al. J. Hep. 2019. Data from 2004-2015. 3. Razavi-Shearer, et. al. JVH, 2023. 4. Based on initial market research data conducted by third party.

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Estimated ~$1B US market opportunity annually with greater potential worldwide sales

Pevifoscorvir sodium Upcoming Milestones

✓ **Jan. 2026:** Planned enrollment (60) completed in HBeAg- subjects

1H2026: 12-week interim phase 2 analysis in approx. 60% (36) of participants (HBeAg-)

2H2026: 24-week interim phase 2 analysis in approx. 50% (55) of subjects (HBeAg+)

2027: Topline phase 2

To preserve study integrity and comply with FDA regulatory requirements, the company will remain blinded to subject-level data and will receive insights from the Data Safety Monitoring Board on protocol-specified assessments, including the potential for sample size re-estimation at each interim analysis

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ALG-055009 for Obesity, MASH

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ALG-055009

A Potential Best-in-Class THR-β Agonist for Obesity, MASH

- **Discovered by Aligos; issued US patent expires 2040[1]**
- **Purpose-built with enhanced pharmacologic properties**
  - ~5-50x fold more potent
  - More β selective
  - Optimized for PK

  } vs. competitor THR-β agonists[3]
- **MASH: Phase 1 highlights**
  - PK - dose proportional, low variability, $t_{1/2}$ ~20 hours (enhanced vs. resmetirom[2])
  - Well tolerated without clinical safety signals
  - Pharmacodynamics - dose proportional increases in SHBG, decreases in atherogenic lipids
- **MASH: Phase 2a HERALD study highlights**
  - Well-tolerated, with rates of GI-related AEs similar to placebo based on data to date
  - Primary endpoint achieved with robust reductions in liver fat content at Week 12 with a low oral dose
  - 11/14 subjects on stable GLP-1 treated with ALG-055009 had liver fat decreases, whereas 4/4 subjects on stable GLP-1 treated with placebo had liver fat increases

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[1] Not including any patent term extension. [2] No head-to-head clinical trials have been conducted. Caution should be exercised when comparing data across trials. [3] Data on file. EC_{50} HEK293T cell-based assay, nM.

THR-β Agonist (ALG-055009) + Incretin Receptor Agonist for Obesity

- **New preclinical and clinical findings from other companies demonstrate that THR-β agonists can significantly enhance weight loss when administered in combination with incretin receptor agonists (RAs) for obesity**
- **The concept of combining THR-β agonists and incretin RAs for obesity treatment has been clinically validated by others**
  - Ascletis combined their less potent, subcutaneously-dosed THR-β agonist (ASC47) with semaglutide in a Phase 1 study in the U.S. for obesity management
  - The combination resulted in over 50% greater weight reduction compared to semaglutide monotherapy
  - Furthermore, combination therapy mitigated rebound body weight gain after discontinuation of treatment
- **Aligos recently generated compelling preclinical data combining our proprietary THR-β agonist, ALG-055009, with incretin RAs in a diet-induced obese (DIO) mouse model**
  - ALG-055009 exhibited profound synergistic effects when used in combination with semaglutide or tirzepatide
  - The combination therapy also demonstrated enhanced antihyperlipidemic effects as compared to monotherapy
- **ALG-055009 warrants further development for obesity, MASH**
  - Phase 2b enabling activities underway; expected completion in 2025
  - Assessing potential Phase 2b clinical trial study designs with KOLs, and plan to consult with the FDA
  - **Discussions with partners underway; evaluating a variety of options to fund continued development**

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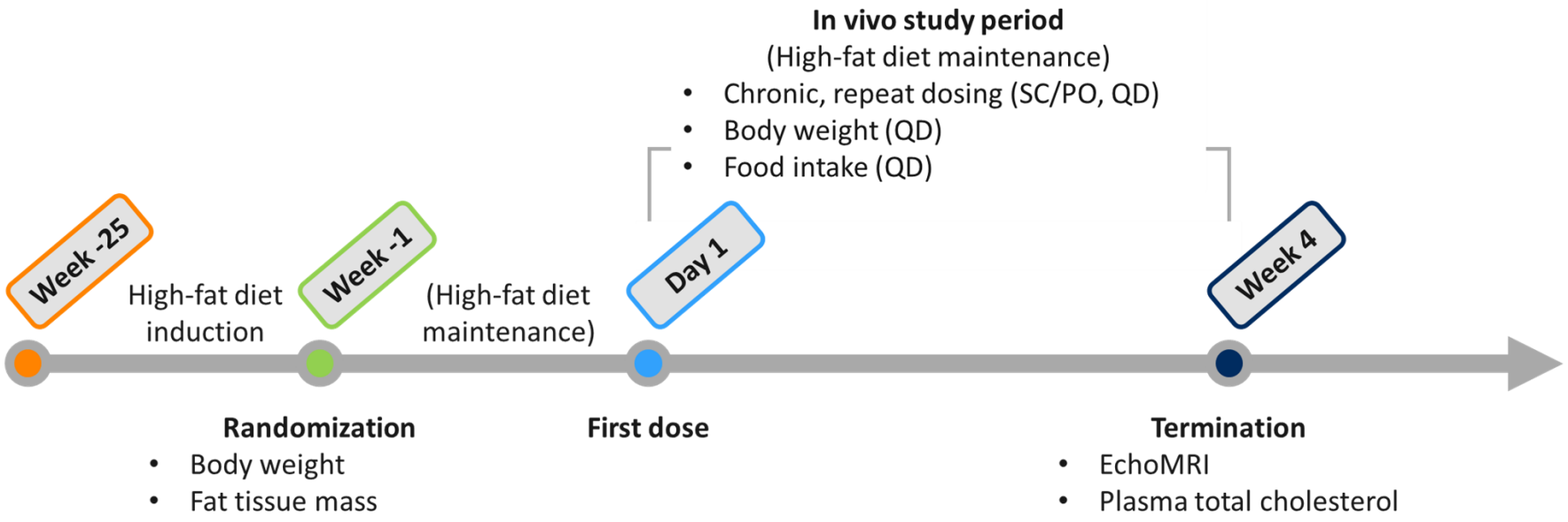
THR-β Agonist (ALG-055009) + Incretin Receptor Agonists

Study Design

- Hypothesis: addition of a THR-β agonist will enhance the magnitude and duration of weight loss effect by GLP-1 RA by attenuating the metabolic adaptation response via normalizing metabolic rate



Treatment groups

1. Vehicle
2. ALG-055009 (PO 1 mg/kg)
3. semaglutide (SEMA, SC 10 nmol/kg)
4. ALG-055009 + SEMA
5. tirzepatide (TIRZEP_lo, SC 3 nmol/kg)
6. ALG-055009 + TIRZEP_lo
7. tirzepatide (TIRZEP_hi, SC 10 nmol/kg)
8. ALG-055009 + TIRZEP_hi

PO =oral; QD =once daily; SC =subcutaneous; RA =receptor agonist.

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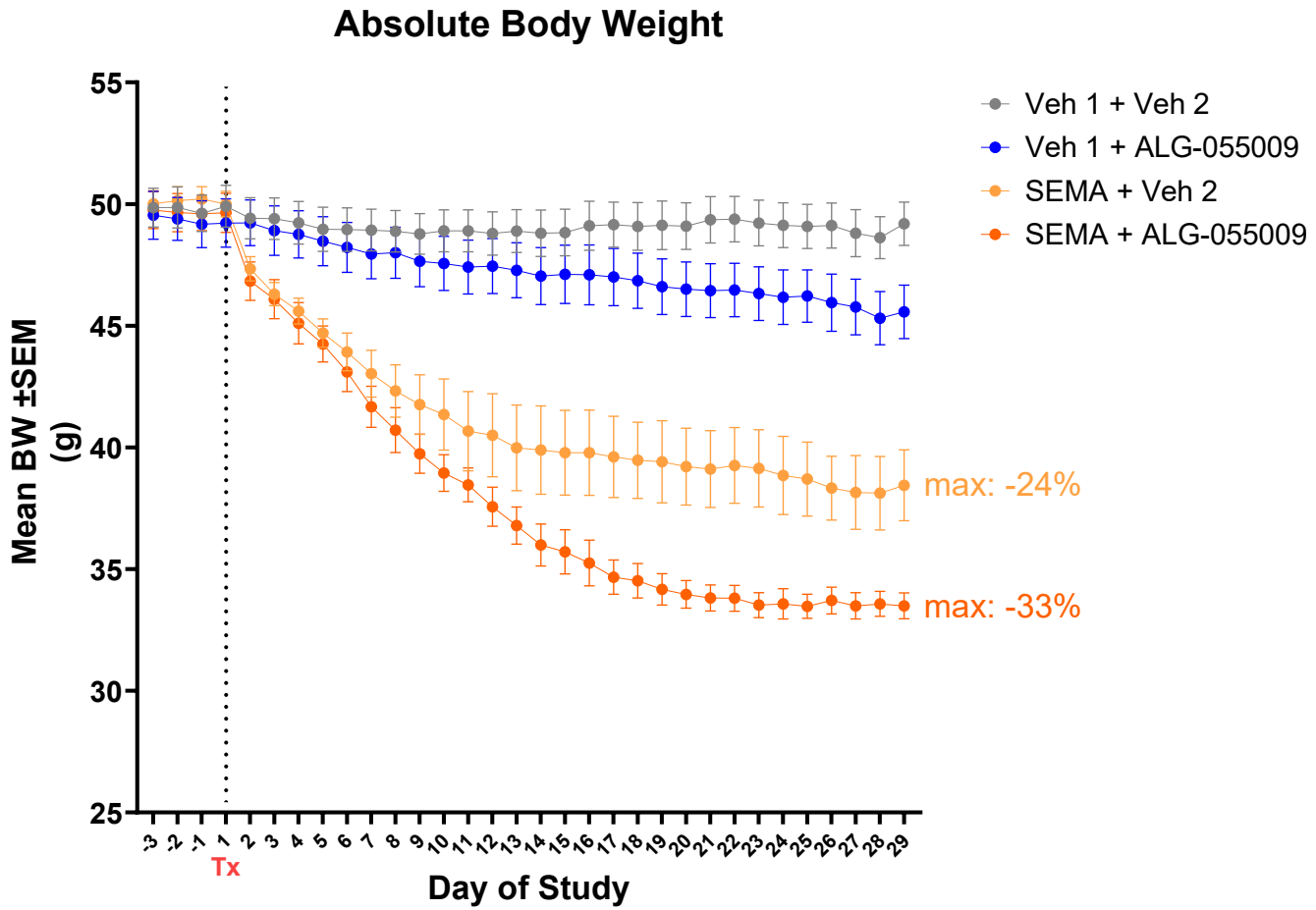
THR-β Agonist (ALG-055009) + Semaglutide

Combination Therapy Enhances Body Weight Loss in Diet-Induced Obese (DIO) Mice



BW = body weight; Tx = start of therapeutic; percentages reported correspond to percent changes in weight as compared to baseline measurements (nadir).

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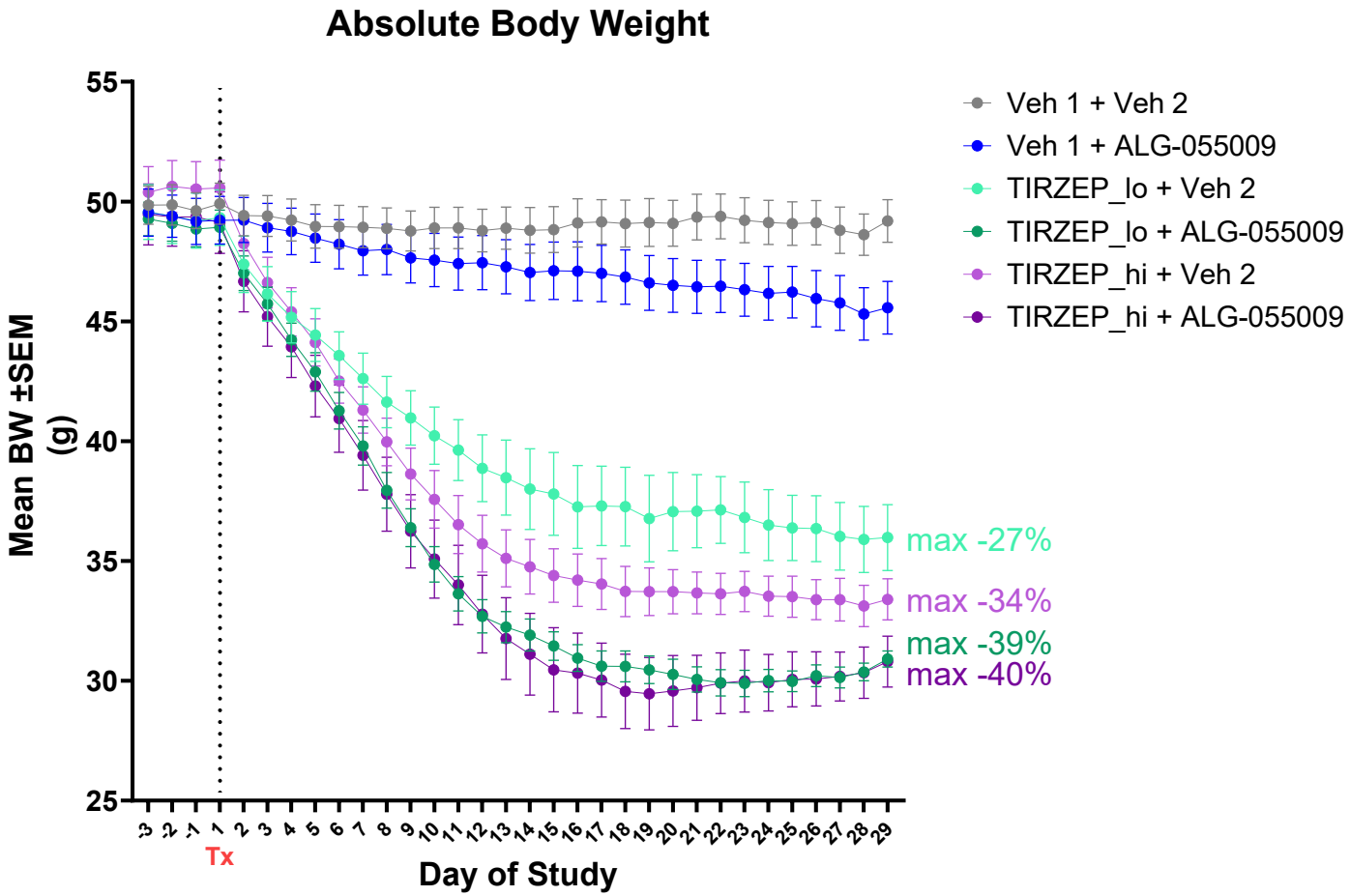
ALG-055009 has profound synergistic effects on BW loss when combined with semaglutide, with as much as 33% decrease in BW within 28 days

THR-β Agonist (ALG-055009) + Tirzepatide

Combination Therapy Enhances Body Weight Loss in Diet-Induced Obese (DIO) Mice



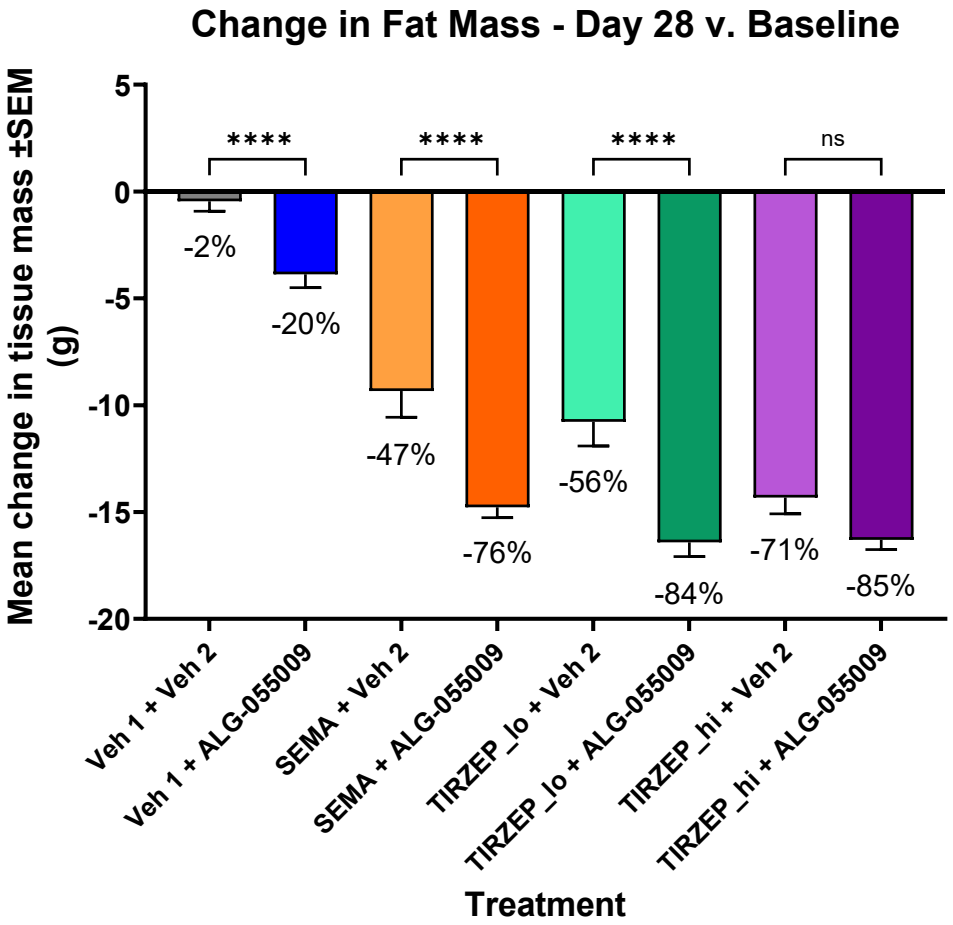
BW = body weight; Tx = start of therapeutic; percentages reported correspond to percent changes in weight as compared to baseline measurements (nadir).

ALIGOS THERAPEUTICS

ALG-055009 has profound synergistic effects on BW loss when combined with both low- and high-dose tirzepatide, with as much as 40% decrease in BW within 28 days

THR-β Agonist (ALG-055009) + Incretin Receptor Agonists

Combination Therapy Enhances Loss of Fat Mass in Diet-Induced Obese (DIO) Mice



One-way ANOVA with Tukey's multiple comparisons test; percentages displayed below each bar correspond to percent changes in tissue mass as compared to baseline measurements; **** = p-value <0.0001; ns = not statistically significant.

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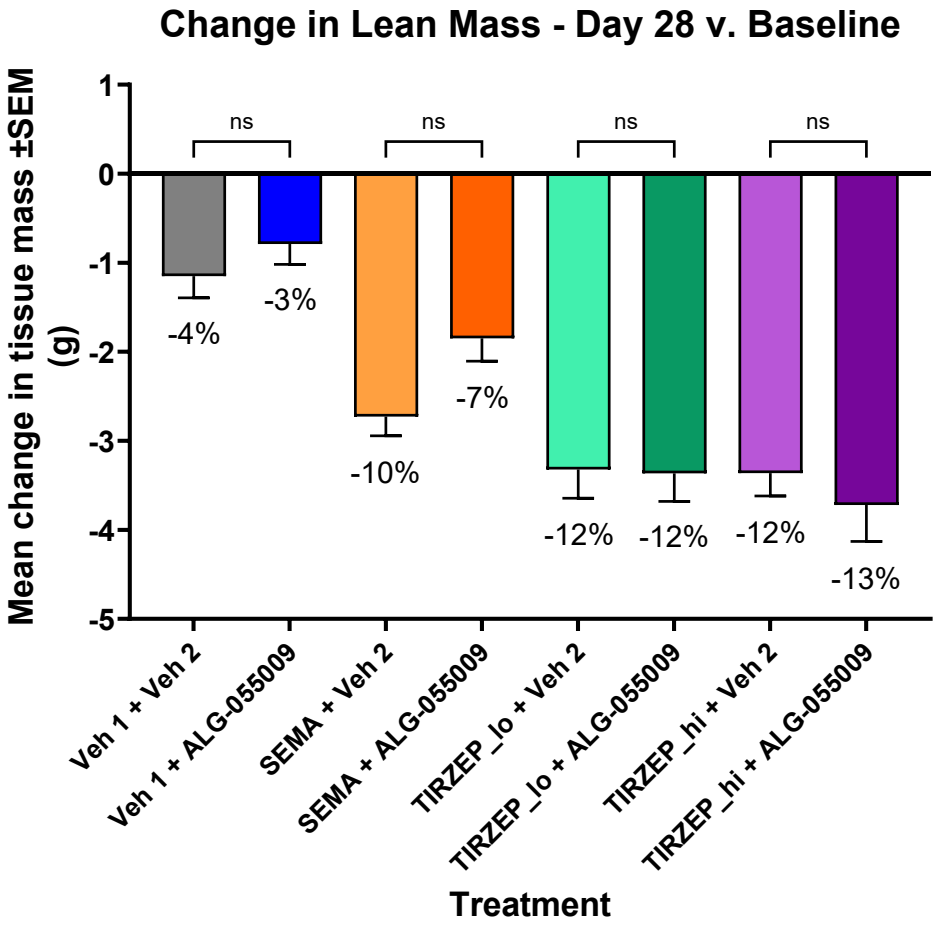
The addition of ALG-055009 to incretin RA treatment results in greater decreases in fat mass as compared to incretin RA monotherapy

THR-β Agonist (ALG-055009) + Incretin Receptor Agonists

Combination Therapy Does Not Affect Changes in Lean Mass in Diet-Induced Obese (DIO) Mice



One-way ANOVA with Tukey's multiple comparisons test; percentages displayed below each bar correspond to percent changes in tissue mass as compared to baseline measurements; ns = not statistically significant.

ALIGOS THERAPEUTICS

The addition of ALG-055009 to incretin RA treatment had no significant effects on changes in lean mass as compared to incretin RA monotherapy

THR-β Agonist (ALG-055009) + Incretin Receptor Agonists

Summary

- DIO mice treated with ALG-055009 in combination with semaglutide or tirzepatide (low or high dose) experience greater weight loss for longer durations compared to animals treated only with incretin RA
  - The additional weight loss conferred by ALG-055009 is mainly due to decrease in fat tissue mass and not due to changes in lean tissue mass or food intake
  - ALG-055009 + SEMA reaches similar weight loss as TIRZEP_hi monotherapy
  - ALG-055009 + TIRZEP_lo has greater weight loss compared to TIRZEP_hi monotherapy indicating a possible dose-sparing regimen
- We provide evidence that the synergism is a result of ALG-055009's ability, as a THR-β agonist, to increase overall metabolism and overcome the metabolic adaptation that emerges after a period of weight loss
- Furthermore, combination therapy exhibited enhanced antihyperlipidemic effects as compared to monotherapy

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